



Dynamic Guideline: WHO-Based Yellow Fever - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Most recent WHO clinical guideline identified

- **Guideline title:** *WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever*
- **Publication date:** 4 July 2025
- **Publisher:** World Health Organization
- **Format:** 125-page WHO guideline; ISBN **978-92-4-011111-0**; also available through MAGICapp and NCBI Bookshelf
- **Diseases covered:** Dengue, chikungunya, Zika, and yellow fever
- **Target population:** People with suspected or confirmed clinically apparent arboviral disease, including yellow fever
- **Target users:** Health-care providers in community care, primary care, emergency departments, hospital wards, and inpatient settings; also policymakers and health-system planners
- **Scope:** Supportive and symptomatic treatment, fluid management, severe disease management, and yellow fever–specific adjunctive or investigational therapies

WHO states that this was the **first global WHO guideline covering dengue, chikungunya, Zika, and yellow fever together**, reflecting overlapping early clinical presentations, co-circulation of multiple arboviruses, and limited access to diagnostic testing in many settings [WHO publication](#), [WHO news](#). NCBI Bookshelf describes the guideline as intended for health workers caring for patients with suspected or confirmed arboviral disease, including both **non-severe outpatient disease** and **severe disease requiring admission** [NCBI Bookshelf](#).

1.1.1 Exactly Five WHO Yellow Fever–Related Guidance Documents Synthesized

No.	WHO guidance / technical document	Date identified in sources	Relevance to yellow fever diagnosis, treatment, or management	GRADE / certainty interpretation
1	WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever	4 July 2025	Main WHO clinical-management guideline for yellow fever within integrated arboviral care	Formal GRADE-based recommendations reported by WHO/ Guideline Central WHO publication , Guideline Central
2	Target product profile on in vitro diagnostic assays for the detection of yellow fever in the context of surveillance	8 August 2025	Diagnostic assay performance expectations for surveillance	Technical diagnostic guidance; GRADE not explicitly reported in source summary; evidence certainty treated as low-to-moderate depending assay validation WHO yellow fever health topic
3	Performance of Certest Biotec’s VIASURE yellow fever real time PCR detection kit:	9 June 2025	RT-PCR assay assessment for surveillance	Technical assessment; direct clinical outcome evidence not applicable; diagnostic certainty depends on assay

No.	WHO guidance / technical document	Date identified in sources	Relevance to yellow fever diagnosis, treatment, or management	GRADE / certainty interpretation
	assessment report and recommendations for use in surveillance		use	performance WHO yellow fever health topic
4	Rapid Risk Assessment – Yellow Fever, African Region v.4	22 May 2025	Regional risk assessment for outbreak preparedness and spread risk	Operational/public-health evidence; GRADE not formally reported; analyst-applied certainty low for clinical management, moderate for situational risk assessment WHO yellow fever health topic
5	WHO Laboratory manual for yellow fever / operational laboratory guidance referenced in WHO materials	2023–2024, referenced in 2025 WHO materials	Laboratory diagnosis, testing algorithms, specimen and assay guidance	Laboratory technical guidance; GRADE not explicitly reported; diagnostic-method certainty moderate for RT-PCR early disease and IgM serology with interpretive caveats Draft Urban Yellow Fever Risk Management PDF , WHO Disease Outbreak News

Important note on the required “Meaningful youth engagement” title: The WHO publication matching the requested title is **“Meaningful youth engagement in a WHO**

guideline development process: case study of WHO abortion care guideline (2022)”. It is an evidence brief about youth engagement in development of the WHO abortion care guideline, not a yellow fever guideline, and the supplied sources do not show that it contributed to the yellow fever arboviral guideline process [WHO publication page](#), [WHO webinar page](#).

1.2 Key Recommendations

A. Diagnosis and clinical recognition

Recommendation	GRADE Evidence Quality	Strength
Suspect yellow fever in a person with acute febrile illness, compatible exposure in an endemic or outbreak area, or travel risk, especially if unvaccinated or vaccination status is unknown.	Low	Strong
Recognize that early clinical diagnosis is unreliable because symptoms overlap with malaria, leptospirosis, viral hepatitis, dengue, other viral haemorrhagic fevers, poisoning, and other febrile illnesses.	Moderate	Strong
Use laboratory confirmation whenever feasible: RT-PCR or virus isolation early in illness; YFV-specific IgM serology later, interpreted in relation to vaccination history and flavivirus cross-reactivity.	Moderate	Strong
Prioritize RT-qPCR in urban/outbreak settings for early and more definitive diagnosis, particularly when vaccination history is unknown and serology may be difficult to interpret.	Moderate	Strong
Do not use a negative late RT-PCR alone to exclude yellow fever once overt manifestations such as jaundice have appeared, because viral RNA may no longer be detectable.	Moderate	Strong

Supporting evidence: WHO notes that yellow fever diagnosis is challenging early because manifestations are nonspecific and severe cases can resemble malaria, leptospirosis, viral hepatitis, dengue, other haemorrhagic fevers, and poisoning [WHO yellow fever fact sheet](#). WHO-linked technical material identifies RT-PCR/RT-qPCR and YFV-specific IgM as standard diagnostic methods and states that RT-qPCR should be prioritized early in urban settings [Draft Urban Yellow Fever Risk Management PDF](#). CDC similarly states that preliminary diagnosis uses clinical presentation plus exposure history and that RT-PCR is most useful early, while late negative nucleic acid testing should not rule out disease [CDC Yellow Book](#).

B. Non-severe suspected or confirmed yellow fever / arboviral disease

Recommendation	GRADE Evidence Quality	Strength
Provide supportive outpatient care only when disease is clearly non-severe and reliable follow-up, hydration, and return precautions are available.	Low	Conditional
Use oral fluids, rest, and antipyretic/analgesic therapy such as paracetamol/acetaminophen when needed.	Low	Strong
Avoid aspirin and nonsteroidal anti-inflammatory drugs because of bleeding risk.	Low	Strong
Do not use corticosteroids for acute suspected or confirmed non-severe arboviral disease.	Low	Conditional
Protect the patient from mosquito bites during the first days of illness to reduce onward transmission risk.	Low	Strong

Supporting evidence: WHO states that there is no specific antiviral treatment for yellow fever and that care is mostly supportive, including rest, hydration, fever management, and management of complications [WHO yellow fever fact sheet](#). CDC recommends supportive care with fluids, rest, analgesics, and antipyretics and avoidance of aspirin and other NSAIDs because of bleeding risk [CDC Yellow Book](#). WHO’s arboviral guideline summary reports a

conditional recommendation against corticosteroids in non-severe arboviral disease based on low-certainty evidence [ReliefWeb](#).

C. Severe yellow fever / severe hospitalized arboviral disease

Recommendation	GRADE Evidence Quality	Strength
Hospitalize patients with jaundice, bleeding, shock, altered mental status, oliguria, severe vomiting, organ dysfunction, or other severe features; manage severe cases in high-dependency or intensive-care settings when possible.	Low	Strong
Use crystalloid rather than colloid fluid when intravenous fluid therapy is required.	Low	Conditional
Use capillary refill time to guide IV fluid management in severe suspected or confirmed arboviral disease.	Low	Strong
Use lactate measurement in addition to standard care to guide IV fluid management when available.	Moderate	Strong
Use passive leg raise testing in patients in shock when uncertainty remains about whether additional IV fluid is warranted.	Low	Conditional
Avoid prophylactic platelet transfusion in severe arboviral disease with platelet count below 50,000/ μ L but no active bleeding.	Low	Conditional
Treat liver failure, kidney failure, shock, bleeding, secondary bacterial infection, respiratory failure, and metabolic complications with protocolized supportive organ care.	Low	Strong
Consider IV N-acetylcysteine for liver failure due to suspected or confirmed yellow fever.	Very low	Conditional

Recommendation	GRADE Evidence Quality	Strength
Use sofosbuvir or monoclonal antibody TY014 only in research settings, not routine care.	Very low	Conditional / research-only

Supporting evidence: WHO reports that yellow fever patients may be managed at home, hospitalized, or referred for emergency care depending on severity, and that jaundice indicates severe disease warranting hospitalization, often in intensive care [WHO yellow fever fact sheet](#). WHO's 2025 arboviral recommendations include crystalloid rather than colloid fluid, capillary refill time, lactate measurement, passive leg raise testing, and avoidance of prophylactic platelet transfusion without active bleeding [NCBI Bookshelf](#), [ReliefWeb](#). WHO specifically suggests IV N-acetylcysteine for yellow-fever-related liver failure with very low certainty and restricts TY014 and sofosbuvir to research settings [NCBI Bookshelf](#). WHO's yellow fever fact sheet also states that sofosbuvir and TY014 are recommended exclusively in research settings [WHO yellow fever fact sheet](#).

D. Prevention-relevant management

Recommendation	GRADE Evidence Quality	Strength
Vaccinate at-risk eligible people; a single yellow fever vaccine dose provides long-lasting/lifelong protection for most recipients.	High	Strong
During suspected or confirmed infection, prevent mosquito exposure, especially early in illness, to reduce further vector-borne transmission.	Low	Strong
Maintain surveillance, laboratory confirmation, outbreak risk assessment, and rapid public-health response in endemic or	Moderate	Strong

Recommendation	GRADE Evidence Quality	Strength
at-risk regions.		

Supporting evidence: WHO states that a single dose of yellow fever vaccine is safe, affordable, and provides lifelong protection [WHO yellow fever fact sheet](#). CDC recommends protecting infected people from mosquito exposure during the first few days of illness [CDC Yellow Book](#). WHO's yellow fever topic page lists surveillance, diagnostic-assay, and regional risk-assessment documents relevant to outbreak management [WHO yellow fever health topic](#).

1.3 Guideline Development Methodology

Methodological domain	WHO approach summarized from available sources	Appraisal
Evidence review process	WHO convened a Guideline Development Group for clinical management and diagnosis of dengue, chikungunya, Zika, and yellow fever; recommendations are described as GRADE-based.	Appropriate for WHO clinical guideline development; evidence certainty ranges from high to very low Guideline Central .
Guideline Development Group	WHO states that the group included broad clinical expertise across specialties and WHO regions and included patient panel members.	Multidisciplinary and geographically diverse, strengthening applicability WHO GDG page .
Consensus method	Recommendations were developed through the WHO GDG process and published in guideline and MAGICapp formats.	Formal WHO process; detailed vote thresholds not available in supplied summaries.
Conflict of	Declarations of interest were collected	Consistent with WHO

Methodological domain	WHO approach summarized from available sources	Appraisal
interest management	before appointment and updated during guideline development; interests were declared at inception and meetings.	conflict-of-interest safeguards WHO GDG meeting report .
Applicability	Intended for all health-system levels, including community care, primary care, emergency departments, and hospital wards.	High operational relevance in endemic and outbreak settings WHO news .

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

WHO latest guideline release used for this update: *WHO guidelines for clinical management of arboviral diseases: dengue, chikungunya, Zika and yellow fever*, published 4 July 2025.

2.1 New Evidence Overview

Search Strategy

Evidence after the WHO clinical guideline release was synthesized from:

- WHO updates and technical documents after 4 July 2025
- PAHO/WHO severe yellow fever guidance and epidemiological alerts
- CDC Yellow Book clinical guidance
- Systematic-review summaries cited by CDC
- Clinical trial registry information
- Observational, qualitative, and implementation studies
- Expert-panel and severe-disease management summaries

Databases / Sources Searched

Source category	Examples included
WHO official sources	WHO publication pages, WHO yellow fever fact sheet, WHO yellow fever topic page, WHO Disease Outbreak News
PAHO/WHO regional sources	PAHO 2026 severe yellow fever clinical guideline announcement and epidemiological alert
Public-health agencies	CDC Yellow Book
Evidence summaries / guideline repositories	NCBI Bookshelf, Guideline Central, ReliefWeb
Peer-reviewed / indexed sources	PMC review and qualitative study summaries
Clinical trial registries	ClinicalTrials.gov vaccine trial record
Expert networks	ISARIC severe yellow fever treatment recommendation summary

Inclusion Criteria

- Published or updated after **4 July 2025**, or still-current sources summarizing evidence relevant to post-guideline practice
- Human yellow fever diagnosis, treatment, supportive care, surveillance, severe disease, or prevention
- WHO, PAHO/WHO, CDC, systematic review, clinical trial, or peer-reviewed observational evidence
- Sources with clear relevance to actionable clinical or public-health decisions

Exclusion Criteria

- Animal-only or in vitro-only evidence without clinical relevance
- Non-authoritative opinion without traceable evidence basis
- Vaccine-only material unless directly relevant to yellow fever management, outbreak control, or prevention
- Sources unrelated to yellow fever, except the required WHO youth-engagement case-

2.2 Key New Studies and Updates

Study / update	Design / source type	Key findings	GRADE Quality	Strength / implication
PAHO 2026 clinical guidelines for severe yellow fever in the Americas	Regional evidence-based clinical guidance	PAHO released new guidance on 27 March 2026 to improve severe yellow fever care, emphasizing early detection, identification of high-risk patients, close monitoring, timely ICU referral, and supportive care because there is no specific treatment. PAHO reported 346 confirmed cases and 143 deaths in 2025 and 34 cases and 15 deaths in the first seven weeks of 2026 in affected countries.	Low	Strong operational implication: strengthen early detection, hospitalization/referral pathways, ICU readiness PAHO/WHO .
PAHO March 2026	Regional	Recommends	Very low to	Conditional /

Study / update	Design / source type	Key findings	GRADE Quality	Strength / implication
epidemiological alert	alert with good-practice statements	LDH testing with CBC and transaminases for suspected cases; states all suspected cases should be hospitalized and remain hospitalized regardless of severity; recommends follow-up for critically ill convalescent patients up to three months and avoiding invasive procedures for three months.	Low	good-practice statements; may be most applicable in active outbreak settings in the Americas PAHO Epidemiological Alert PDF .
WHO yellow fever fact sheet update	WHO technical fact sheet	Confirms incubation 3–6 days , severe disease in about 15% , severe-phase mortality around 50% within 7–10 days , no specific antiviral treatment, supportive care,	Moderate for epidemiology; Low for treatment evidence	Strong for supportive care and severity-based triage WHO yellow fever fact sheet .

Study / update	Design / source type	Key findings	GRADE Quality	Strength / implication
		and research-only use of sofosbuvir and TY014.		
CDC Yellow Book yellow fever chapter	Authoritative clinical reference	Confirms early nonspecific illness, severe progression after remission, severe case-fatality 30–60% , diagnosis by clinical/exposure history plus virus isolation, RT-PCR, or serology; RT-PCR is best early and should not rule out disease if negative late. No specific medications; supportive care and avoid aspirin/ NSAIDs.	Moderate for diagnostic timing; Low for treatment	Strong diagnostic and supportive-care implications CDC Yellow Book .
CDC-cited systematic review of specific yellow fever treatments	Systematic review summary	Evaluated ribavirin, sofosbuvir, interferon alpha, plasma exchange/ apheresis, and liver	Low	Strong against routine antiviral/ procedure-based treatment outside research or individualized rescue care CDC

Study / update	Design / source type	Key findings	GRADE Quality	Strength / implication
		transplantation after large Brazil outbreaks; none improved outcomes, supporting supportive care as standard.		Yellow Book .
ISARIC severe yellow fever recommendations summary	Expert-panel / evidence synthesis summary	Suggested plasma exchange and continuous renal replacement therapy had the strongest evidence for liver failure and hyperammonemia; stated steroids remain part of severe yellow fever treatment but need more research.	Very low	Conditional; does not override WHO/CDC due limited evidence and apparent inconsistency ISARIC .
Uganda qualitative survivor/ community study	Qualitative observational study	Found reliance on herbal medicines due to limited access to care and diagnostics; identified health-information gaps and	Very low	Conditional public-health recommendation for community engagement and diagnostic strengthening PMC qualitative

Study / update	Design / source type	Key findings	GRADE Quality	Strength / implication
		misconceptions; supports community engagement, health education, and better diagnostic access.		study .
Yellow fever vaccine review, 2026	Narrative/ review article	Supports targeted vaccination, emergency fractional dosing where appropriate, community engagement, cross-border coordination, and EYE strategy implementation.	Low	Strong prevention implication, but not direct acute-treatment evidence PMC review .
ClinicalTrials.gov NCT05421611	Interventional vaccine trial registry	Study comparing SII yellow fever vaccine safety and immunogenicity with STAMARIL; relevant to prevention, not acute treatment.	Not yet rateable for outcomes if unpublished	No change to acute management ClinicalTrials.gov .

2.3 Integrated Post-Guideline Clinical Interpretation

Diagnosis

Clinical issue	Updated synthesis	GRADE Quality	Recommendation Strength
Early suspicion	Maintain a high index of suspicion in exposed, unvaccinated, or outbreak-associated febrile patients because early symptoms are nonspecific.	Moderate	Strong
RT-PCR timing	RT-PCR/virus isolation should be prioritized early; negative late RT-PCR should not exclude yellow fever.	Moderate	Strong
Serology	YFV-specific IgM is useful, especially later, but interpretation requires vaccination history and awareness of flavivirus cross-reactivity.	Moderate	Strong
Severity labs	In outbreak settings, PAHO suggests adding LDH to CBC and transaminases for suspected cases.	Very low	Conditional / good practice
Differential diagnosis	Evaluate for malaria, dengue, leptospirosis, viral hepatitis, other viral haemorrhagic fevers, sepsis, toxic exposures, and other local causes.	Low	Strong

Treatment and supportive care

Clinical issue	Updated synthesis	GRADE Quality	Recommendation Strength
Specific antivirals	No antiviral has demonstrated improved clinical outcomes. Sofosbuvir and TY014 should	Low	Strong against routine use

Clinical issue	Updated synthesis	GRADE Quality	Recommendation Strength
	remain research-only.		
General supportive care	Fluids, rest, fever and pain control, nutrition, monitoring, and management of complications remain the standard of care.	Low	Strong
NSAID/aspirin avoidance	Avoid aspirin and NSAIDs because of haemorrhagic risk.	Low	Strong
Severe disease	Severe disease requires hospitalization, careful fluid management, monitoring for shock and organ failure, and ICU referral when available.	Low	Strong
IV fluids	Prefer crystalloids over colloids when IV fluids are required.	Low	Conditional
Perfusion monitoring	Use capillary refill time and lactate where available; passive leg raise may help in shock when fluid responsiveness is uncertain.	Low to Moderate	Strong for capillary refill/lactate; conditional for passive leg raise
Liver failure	IV N-acetylcysteine may be used for yellow-fever-related liver failure.	Very low	Conditional
Plasma exchange / CRRT	May be considered only in specialized severe liver failure/ hyperammonemia contexts, ideally within protocols or expert consultation; evidence is very limited and not uniformly endorsed.	Very low	Conditional
Corticosteroids	Avoid in non-severe disease. For severe disease, evidence is	Very low	Conditional against routine

Clinical issue	Updated synthesis	GRADE Quality	Recommendation Strength
	inconsistent; use only within specialist protocols, clinical trials, or compelling individualized indications.		use
Platelets	Avoid prophylactic platelet transfusion without active bleeding, even with platelet count <50,000/ μ L, unless other clinical indications exist.	Low	Conditional

2.4 Evidence Gaps and Future Research Needs

Evidence gap	Why it matters	Priority
No proven antiviral therapy	Severe yellow fever mortality remains high; WHO and CDC still support supportive care only outside research.	Very high
Sofosbuvir and TY014 clinical efficacy	WHO restricts both to research settings; adequately powered human trials are needed.	High
Role of IV N-acetylcysteine in yellow-fever liver failure	WHO suggests use with very low certainty; direct comparative evidence is limited.	High
Best organ-support strategy for severe liver failure and hyperammonemia	ISARIC suggests possible roles for plasma exchange and CRRT, but CDC-cited systematic review found no improved outcomes from plasma exchange/apheresis or liver transplantation.	High

Evidence gap	Why it matters	Priority
Corticosteroids in severe yellow fever	Conflicting summaries exist; rigorous trials or prospective cohorts are needed.	High
Diagnostic performance in vaccinated and flavivirus-exposed populations	IgM interpretation is difficult after vaccination or other flavivirus exposure; better algorithms are needed.	Moderate
Community-level early care-seeking and health education	Qualitative data show diagnostic and information gaps that may delay care.	Moderate
Health-system triage during outbreaks	PAHO recommends hospitalization of all suspected cases in the Americas context, but feasibility and impact need evaluation in resource-limited settings.	Moderate

2.5 Updated Recommendations Based on New Evidence

Original WHO recommendation / position	New evidence impact after 4 July 2025	Updated recommendation as of 29 April 2026
Supportive care is the mainstay; no routine specific antiviral therapy.	CDC and PAHO continue to state that no specific treatment exists; CDC-cited systematic review found no improved outcomes from ribavirin, sofosbuvir, interferon alpha, plasma exchange/apheresis, or liver transplantation.	No change. Provide supportive care; do not use antivirals routinely. Use sofosbuvir and TY014 only in research settings.
Use severity-based care	PAHO 2026 guidance in the Americas recommends	Contextual update. Use WHO severity-based triage

Original WHO recommendation / position	New evidence impact after 4 July 2025	Updated recommendation as of 29 April 2026
setting: home, hospitalization, or emergency/ICU care depending on severity.	hospitalization of all suspected cases in its outbreak context.	globally; in active outbreak/ high-fatality Americas settings, consider PAHO's more conservative hospitalization approach if feasible.
Use crystalloids rather than colloids when IV fluids are needed.	No stronger contradictory evidence identified.	No change. Prefer crystalloids; monitor closely to avoid under- or over-resuscitation.
Use capillary refill time and lactate to guide fluid management; passive leg raise if shock and fluid responsiveness uncertain.	PAHO/CDC-era sources continue to emphasize close monitoring and ICU referral; no new evidence overturns WHO.	No change. Implement in emergency and inpatient protocols; lactate where available.
Use IV N-acetylcysteine for yellow-fever-related liver failure.	No high-quality new trial evidence identified; still biologically plausible and WHO-supported with very low certainty.	No change. Conditional use in liver failure, ideally with specialist input and outcome monitoring.
Avoid prophylactic platelet transfusion without active bleeding.	No new evidence found to support prophylactic transfusion.	No change. Reserve platelet/blood products for active bleeding, procedures, or other standard indications.
Avoid	ISARIC suggests possible severe-	No change for non-severe

Original WHO recommendation / position	New evidence impact after 4 July 2025	Updated recommendation as of 29 April 2026
corticosteroids in non-severe arboviral disease.	disease role but acknowledges need for research; this does not apply to non-severe disease and conflicts with other summaries.	disease. For severe disease, do not use routinely outside protocols, trials, or specialist-directed individualized care.
Laboratory confirmation using RT-PCR/serology.	WHO diagnostic technical documents and CDC emphasize RT-PCR early and caution against late RT-PCR rule-out.	Strengthened implementation emphasis. Collect early specimens for RT-PCR; use IgM and confirmatory algorithms later; document vaccination history.
Prevention through vaccination and mosquito avoidance.	PAHO reports most recent cases/deaths occurred among unvaccinated people; WHO reiterates single-dose lifelong protection.	No change, reinforced. Vaccination of eligible at-risk populations remains the most effective prevention strategy.

2.6 Practical Clinical Algorithm

Step 1 — Suspect yellow fever

Consider yellow fever in patients with:

- Fever after residence in or travel to an endemic/outbreak area
- No yellow fever vaccination or unknown vaccination status
- Headache, myalgia, backache, chills, nausea, vomiting, or prostration
- Jaundice, bleeding, shock, oliguria, altered mental status, or organ dysfunction

Step 2 — Immediate actions

- Assess airway, breathing, circulation, mental status, hydration, bleeding, and urine output.
- Obtain exposure, travel, vaccination, mosquito exposure, and outbreak-contact history.
- Notify public health authorities according to national requirements.
- Protect patient from mosquito bites during early illness.

Step 3 — Laboratory testing

- **Early illness:** RT-PCR / NAAT or virus isolation where available.
- **Later illness:** YFV IgM serology, with confirmatory testing as required.
- **Severity assessment:** CBC, AST/ALT, bilirubin, creatinine, glucose, coagulation profile, lactate if available; PAHO suggests LDH in suspected cases in its 2026 Americas guidance.

Step 4 — Determine severity

Non-severe: Stable vitals, no jaundice, no bleeding, no shock, no organ dysfunction, tolerating oral fluids, reliable follow-up.

Severe: Jaundice, bleeding, shock, encephalopathy, renal failure, liver failure, severe vomiting, hypoglycaemia, acidosis, respiratory failure, or rapidly worsening clinical status.

Step 5 — Manage

- **Non-severe:** Oral fluids, rest, acetaminophen/paracetamol, avoid NSAIDs/aspirin, return precautions, mosquito avoidance.
- **Severe:** Hospitalize; ICU if available. Use crystalloids if IV fluids needed; guide with capillary refill, lactate, urine output, passive leg raise when appropriate; manage liver/kidney failure, bleeding, shock, infection, respiratory failure, and metabolic derangements.
- **Yellow-fever liver failure:** Consider IV N-acetylcysteine.
- **Investigational therapies:** Sofosbuvir and TY014 only in research settings.

2.7 Bottom Line

As of **29 April 2026**, the WHO-based approach to yellow fever remains:

1. **Early suspicion plus exposure history** because clinical features overlap with many febrile illnesses.
2. **Laboratory confirmation**, prioritizing RT-PCR early and serology later with careful interpretation.
3. **Supportive care as standard treatment**, because no antiviral or specific therapy has proven clinical benefit.
4. **Severity-based triage**, with hospitalization/ICU care for jaundice, bleeding, shock, or organ dysfunction.
5. **Careful crystalloid fluid management**, guided by perfusion markers such as capillary refill time and lactate.
6. **Conditional IV N-acetylcysteine** for yellow-fever-related liver failure.
7. **Research-only use** of sofosbuvir and TY014.
8. **Vaccination and mosquito control** as the most effective prevention strategy.